

Monitoring Manager Assignment

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1 Scope and method

I used R so the work can be checked, repeated, and updated if the source files change.

Data sources:

- Average_sessions_details.xlsx
- CCP_Monitoring_Data.xlsx

Assignment instructions:

- Monitoring Manager Assignment.pdf

2 Data problems and fixes

Question: What problems are in the two files, and what did I do about them?

Table: Problems found and how they were handled

issue	Frequency	Improvements
Misspelled header	1	Fixed the spelling in the column name; values stayed the same
Location spelling/punctuation variants	28	Made spelling, spacing, and punctuation consistent
City-vs-district geography	4	Kept the city rows; used parent districts only when comparing files
Facility-name formatting/location variants	36	Fixed spacing, commas, and clear location spelling errors
Facility categories vs brief	9	Kept all 9 types and noted that the brief says 8
District facility-count disagreement	59	Kept both counts and marked the districts that need checking
Unspecified averaging window	658	Used the given monthly averages and clearly stated the limitation
Unusual monitoring values	4	Kept the values and marked them for checking
Unusual historical values	20	Kept the values and marked them for checking

The 14 basic format and formula checks found no failures. The monitoring file has 68 reporting areas. For the cross-file comparison only, I grouped 4 city rows with their parent districts, leaving 64 comparable districts. Both files cover 658 facilities in total, but 59 of those districts have different facility counts. I kept both counts because the files do not show which one is current.

Table: Location names that were fixed

source	field	original	corrected
Monitoring	Location	Chattogram / Cox's Bazar	Chattogram / Cox's Bazar
Monitoring	Location	Dhaka / Narshingdi	Dhaka / Narsingdi
Historical	Location	Barishal / Jhalakathi	Barishal / Jhalokathi
Historical	Location	Rangpur / Thakurgaong	Rangpur / Thakurgaon
Historical	Location	Rajshahi / Bogra	Rajshahi / Bogura
Historical	Location	Chattogram / Laxmipur	Chattogram / Lakshmipur
Historical	Location	Barisal / Pirojpur	Barishal / Pirojpur
Historical	Location	Barisal / Jhalokathi	Barishal / Jhalokathi
Historical	Location	Barisal / Barguna	Barishal / Barguna
Historical	Location	Barisal / Patuakhali	Barishal / Patuakhali
Historical	Location	Dhaka / Sariatpur	Dhaka / Shariatpur
Historical	Location	Barisal / Bhola	Barishal / Bhola

Table: Examples of facility names that were fixed

row	original	corrected
56	Adhunik District Sadar Hospital, Khagrachari	Adhunik District Sadar Hospital, Khagrachhari
130	Belabo Upazila Health Complex, Narshingdi	Belabo Upazila Health Complex, Narsingdi
131	Monohardi Upazila Health Complex, Narshingdi	Monohardi Upazila Health Complex, Narsingdi
132	Palash Upazila Health Complex, Narshingdi	Palash Upazila Health Complex, Narsingdi
133	Raipura Upazila Health Complex, Narshingdi	Raipura Upazila Health Complex, Narsingdi
134	Shibpur Upazila Health Complex, Narshingdi	Shibpur Upazila Health Complex, Narsingdi
153	Ghatail Upazila Health Complex,Tangail	Ghatail Upazila Health Complex, Tangail
154	Kalihati Upazila Health Complex,Tangail	Kalihati Upazila Health Complex, Tangail
155	Nagarpur Upazila Health Complex,,Tangail	Nagarpur Upazila Health Complex, Tangail
156	Mirzapur Upazila Health Complex,Tangail	Mirzapur Upazila Health Complex, Tangail

There are 36 facility-name fixes in total. They cover line breaks, apostrophes, commas, spacing, and clear district spelling errors. I did not change other words because there is no official facility list in the files.

Table: Values that should be checked

source	row	location	indicator	value
Monitoring	17	Chattogram / Noakhali	Sessions vs target %	44.2
Monitoring	27	Dhaka / Narayanganj	Sessions vs target %	39.6
Monitoring	54	Rangpur / Dinajpur	Sessions vs target %	38.0
Monitoring	19	Dhaka / Dhaka	Sessions vs target %	35.7
Historical	222	Barishal / Patuakhali	Average monthly sessions	47.8
Historical	404	Khulna / Kushtia	Average monthly sessions	39.3
Historical	555	Rajshahi / Bogura	Average monthly sessions	34.0
Historical	142	Dhaka / Munshiganj	Average monthly sessions	33.1
Historical	107	Mymensingh / Sherpur	Average monthly sessions	31.2
Historical	379	Chattogram / Brahmanbaria	Average monthly sessions	28.0
Historical	41	Mymensingh / Mymensingh	Average monthly sessions	27.6
Historical	235	Rangpur / Dinajpur	Average monthly sessions	27.4

The full review table contains 24 flags. These values look unusual compared with similar rows, but unusual does not always mean wrong. I kept every value and marked it for checking.

Table: Estimated number of months behind the historical averages

active_since	facilities	minimum_implied_months	median_implied_months	maximum_implied_months
2022	20	22	31.5	41
2023	37	8	22.0	30
2024	338	2	12.0	19
2025	170	1	5.0	9
2026	93	1	4.0	7

The session and participant averages use the same number of months for every facility, which is a good sign. However, facilities with the same start year often have very different month counts. Since the file gives only a year, I used the monthly averages as provided and did not guess the missing dates.

3 A. What the numbers show

Main question: What do the numbers tell us about performance, where do the two files disagree, and where should managers focus?

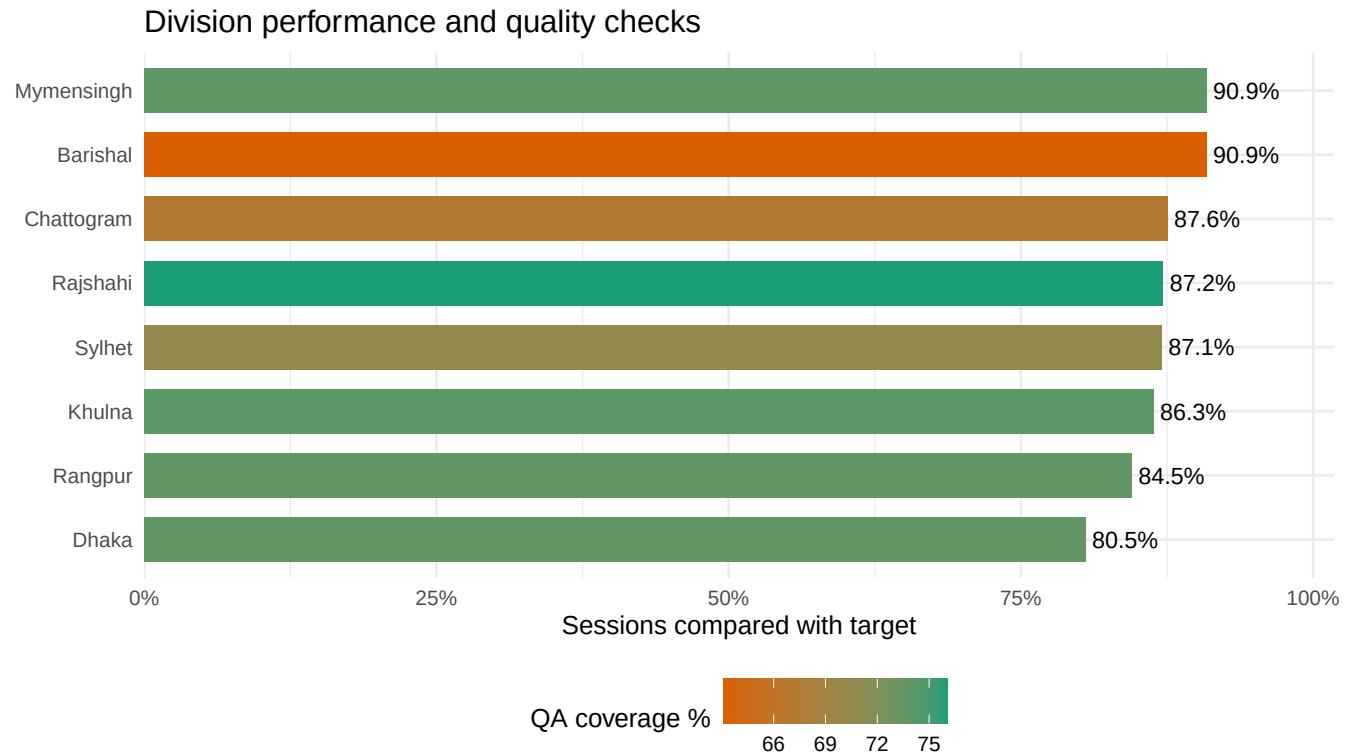
3.1 Division performance

Question 1: How do the eight divisions rank when districts with more sessions carry more weight?

Table: Division ranking from strongest to weakest

Rank	Division	Facilities	Sessions	Participants	Weighted target %	Weighted QA %
1	Mymensingh	30	500	1851	90.9	73.9
2	Barishal	68	1624	5553	90.9	63.1
3	Chattogram	128	2378	8490	87.6	67.4
4	Rajshahi	88	1784	6360	87.2	76.1
5	Sylhet	36	628	1973	87.1	70.5
6	Khulna	96	1938	6927	86.3	74.1
7	Rangpur	67	1328	4537	84.5	73.8
8	Dhaka	145	2428	8664	80.5	73.8

The target ranking uses session volume as the weight, exactly as requested. This means a district running many sessions has more influence than one running only a few. Mymensingh and Barishal both round to 90.9%; Mymensingh is only slightly higher, so that difference is not meaningful on its own. I could not calculate `total sessions / total target` because district target counts were not provided. For the separate QA measure, I used facility count as the weight because QA coverage is tied more closely to facility coverage; the file does not provide the original QA numerator.



3.2 Five-district source comparison

Question 2: For five districts, how does this quarter compare with a three-month estimate based on past monthly averages?

Table: Reported sessions compared with the three-month estimate

District	Monitoring sites	Older-file sites	This quarter	3-month estimate	Difference	Assessment
Gopalganj	8	6	95	74.7	27.2%	Does not align
Naogaon	7	12	144	279.8	-48.5%	Does not align
Rangamati	11	13	184	179.2	2.7%	Totals close; lists differ
Habiganj	10	10	227	163.0	39.3%	Does not align
Barguna	21	9	311	216.9	43.4%	Does not align

I chose one district from each fifth of the session-volume range, so the sample is not made up only of large or small districts. The estimate is the sum of each facility's average monthly sessions multiplied by three. I used a 20% difference as a review flag, not as a pass mark, because the files do not provide normal month-to-month variation. On that basis, 4 of the five districts do not align. The remaining district has a similar session total but a different facility count, so all five need at least a quick check of reporting dates and active-facility lists before anyone treats the gaps as performance changes.

- **Gopalganj (Does not align):** Monitoring includes 2 more facilities, which probably explains part of the higher total. Check the active-facility list and start dates first.
- **Naogaon (Does not align):** Monitoring includes 5 fewer facilities, which probably explains part of the lower total. Check inactive facilities and missing reports first.
- **Rangamati (Totals close; lists differ):** The totals are close, but monitoring lists 11 facilities and the older file lists 13. The close result may be accidental, so check both facility lists and reporting dates.
- **Habiganj (Does not align):** Facility counts match, so the number of facilities does not explain the higher total. Check whether activity grew, the periods differ, or the older average is no longer current.
- **Barguna (Does not align):** Monitoring includes 12 more facilities, which probably explains part of the higher total. Check the active-facility list and start dates first.

3.3 Where managers should focus

Question 3: Which three divisions need the most attention, and what should managers see on a dashboard?

Table: Three divisions that need the most attention

Division	Target %	QA %	Sessions/facility	Facilities/trainer	Attention score
Dhaka	80.5	73.8	16.7	2.4	65.5
Chattogram	87.6	67.4	18.6	2.5	55.9
Barishal	90.9	63.1	23.9	3.1	48.3

I used five practical signals: missed targets (30%), low QA coverage (30%), fewer sessions per facility (15%), more facilities per trainer (15%), and the number of facilities affected (10%). Program size has only a small weight, so it adds the likely reach of a problem without hiding poor performance in a smaller division. The weights are a management choice, not an official standard, and sessions per facility may also reflect the mix of facility types. The score is therefore a shortlist for discussion, not a final judgment.

What I would do in the next 30 days

- **Dhaka — why now:** Largest program (145 facilities), lowest target result (80.5%), and 16.7 sessions per facility. **First action:** Start with the lowest-target districts, check session totals against registers, and agree a six-week recovery plan with weekly follow-up.
- **Chattogram — why now:** Large program (128 facilities), 87.6% target result, and low QA coverage (67.4%). **First action:** Start QA follow-up in districts below 60% coverage and pair every check with coaching, an owner, and a due date.

- **Barishal — why now:** Lowest QA coverage (63.1%) and highest trainer load (3.1 facilities per trainer), despite strong session delivery. **First action:** Protect the strong session volume while increasing QA checks and rebalancing or clustering trainer support.

3.4 Dashboard for the Implementation Team

Visual	One decision it should support
Division scorecard showing target, QA, sessions per facility, trainer load, and facility count	Which divisions need management attention first, and what is driving the concern?
District chart with target result on one axis, QA coverage on the other, and session volume shown by dot size	Is a district's main problem delivery, quality checks, or both?
Stacked bars showing session topics by division	Which division needs help restoring a missing or unusually small session topic?
Data-check table showing missing reports, unusual values, and differences between the two files	Which figures must be checked before a manager acts on them?
Monthly facility activity grid, once monthly facility data are stored	Which facilities have stopped, slowed down, or failed to report for two months?

The first four views can be built from the current files. The monthly facility grid cannot; it needs facility-level reports to be saved by month from now on. Filters should show only fields that exist for the selected view. Clicking a district should open its supporting figures, the action owner, due date, and latest update.

3.5 Executive summary to the Country Director

Question 4: What should the Country Director know before the leadership meeting?

Dear Country Director, this quarter's data shows meaningful differences across the eight divisions. Mymensingh ranks first and Dhaka ranks last when district target results are weighted by session volume. Mymensingh and Barishal are almost tied at 90.9%, so small rank differences should not drive decisions. I recommend first attention to Dhaka, Chattogram, Barishal. Dhaka combines the lowest target result with the largest number of facilities and low sessions per facility. Chattogram has many facilities, below-target delivery, and low QA coverage. Barishal is delivering sessions strongly, but it has the lowest QA coverage and the most facilities per trainer. The monitoring file contains 68 reporting areas. After grouping four city areas with their parent districts only for comparison, there are 64 comparable districts. Both files total 658 facilities, yet 59 districts have different facility counts. The brief also says eight facility types, while the older file contains nine. In the five-district check, Gopalganj, Naogaon, Habiganj, Barguna differ from the three-month estimate by more than 20%. Rangamati's totals are close, but its facility counts still differ. These findings may reflect different facility lists or reporting periods, so they need checking before being treated as performance changes. Over the next 30 days, I would confirm the active-facility lists and reporting dates, start a six-week recovery review in Dhaka, close the largest QA gaps in Chattogram and Barishal, and use a monthly dashboard with named owners and due dates.

Word count: 235.

4 B. Monitoring the 15 remote facilities

Main question: How should Noora Health monitor 15 hard-to-reach facilities with poor internet, long travel times, and frequent nurse transfers?

4.1 Monitoring approach

Question 1: What should be tracked, how should data be collected, how often should staff visit, and what are the main risks?

I would keep the routine list to five measures so it is useful without creating too much work for nurses.

Indicator	Why it matters
Sessions delivered against the monthly plan	Shows whether CCP activity is happening at the expected pace.
Patients and caregivers reached, reported separately	Shows reach and whether one participant group is being missed.
Number of trained nurses still working, including a named backup	Gives an early warning when a transfer could stop delivery.
Short session-quality score from a five-item observation checklist	Checks whether sessions are useful, not only frequent.
Complete report received by the agreed date	Separates a service problem from a reporting or connectivity problem.

The facility nurse should record activity in a numbered paper register as the source record, then enter one facility-month total into Noora Health’s approved offline form. The form should save on the device, check required fields and unusual values, use a facility-and-month ID to prevent repeat submissions, and upload when a signal is available. SurveyCTO’s documentation confirms that its mobile app can collect offline and send data later. A receipt should confirm a successful upload; staff should not resend a form unless no receipt appears. Names or identifiable register photos should not be sent through WhatsApp.

The monitoring officer should make an **initial visit within four to six weeks of launch**, then visit each facility **once per quarter**. An officer covering five facilities would therefore have five planned visits each quarter, usually one or two a month, which leaves time for remote checks and weather delays. Visits can be grouped only when routes and safety allow. During the monthly call, the facility contact should read the aggregate totals from the paper register so the officer can compare them with the submitted form; each visit should include a small source-record check and a live session observation. This is sufficient for routine oversight because direct checks happen every quarter while the monthly calls and warning rules identify problems earlier. One missed deadline should trigger a call; two missed reports, an unexplained zero, a major change, loss of trained staff, or a poor quality score should trigger an earlier visit. This monthly review of completeness, timeliness, unusual changes, and source records follows the practical checks in the WHO routine health facility data toolkit.

Expansion-specific risk	Mitigation
Trained nurses transfer after 6–9 months	Train a main and backup nurse at launch, require a short handover, and give replacements a remote orientation within two weeks.
Poor internet or an app failure causes missing or repeated records	Keep the paper register as the source, use an offline form with a facility-month ID, confirm upload, and compare totals every month.
Weather and long travel delay support or create safety risks	Set quarterly routes with backup dates, use a local contact and check-in plan, avoid unsafe solo travel, and use warning signs to decide when an extra visit is worth the journey.

4.2 Support for the monitoring officer

Question 2: How would I support one officer covering five facilities without asking them to travel too often?

I would agree a three-month route plan with the officer, provide the travel budget in advance, and protect one day each week for remote review and follow-up. Each facility would name a main and backup contact, so the officer can hold one short monthly call and solve routine problems without travelling. A one-page warning list would show

missed deadlines, zero or sharply changing sessions, staff transfers, and open actions; one warning prompts a call, while repeated or serious warnings prompt a visit. For safety, the officer would not make risky river or overnight journeys alone, and every trip would have a weather check, local contact, and check-in/check-out plan. I would meet the officer every two weeks, review a small sample of register totals remotely, arrange cover during leave, and join an occasional visit to coach rather than police.

5 C. Fixing the draft monitoring plan

Main question: What is weak in the draft plan, what could go wrong, and how should the plan be rewritten?

5.1 Specific weaknesses and risks

Question 1: What are the main weaknesses, and what risk does each one create?

Weakness	Concrete risk
Only sessions and participants are tracked	High volume can hide poor session quality, missing topics, late reports, or the loss of trained staff.
Free-text WhatsApp messages have no fixed definitions, record ID, or receipt	Messages can be missed or repeated, and staff may count the same activity in different ways.
Program Managers receive the figures, but the plan does not name a separate person to check the data	Follow-up and data checking can become mixed together, and corrections may not be recorded consistently.
The live master spreadsheet is shared with the research team	Extra access, competing edits, and different definitions can create privacy, version, and consent problems.
Review happens only each quarter, and only zero sessions are flagged	Missing reports, low quality, sudden changes, and gradual decline may remain open for months.

5.2 Rewritten plan (4 sentences)

Question 2: What should the improved plan say?

Each facility will record sessions in a standard register and submit a short offline monthly form by the fifth working day, using fixed definitions and a facility-month ID. The form will check required fields, impossible values, sudden changes, and repeat submissions; the Monitoring Team will compare flagged figures with the source register and record every correction. Program Managers will own follow-up actions, while the Monitoring Team will own data checks and a monthly review of missing reports, zero sessions, low quality, and sharp changes. The research team will receive only a dated, approved copy containing the fields needed for the study, with agreed access and consent rules—not the live program file.

6 D. Lessons from the field notes

Main question: What common lessons appear in the eight field notes, and what should the program change?

6.1 Themes

Question 1: How can the eight notes be grouped, and does each theme look isolated or wider than one site?

Theme	What this tells us	How I would handle it
Materials do not always fit	Standard content is not being checked often enough against the ward setting and information families already receive.	Treat as a wider design risk: two different comments point to poor fit, so check a small sample of facilities before revising the materials.
Timing and access need flexibility	One timetable and one audience rule cannot fit daily ward routines, Ramadan, and different family needs.	Treat as a wider program issue: three notes point to the same design problem; test better time slots and, where privacy allows, a suitable option for interested family members.
Staff changes break continuity	Training depends too heavily on individual nurses instead of a backup and handover system.	Treat as a wider program issue: high turnover is already expected and the field note shows its effect, so every facility needs a trained backup and quick replacement support.
Problems are reported but not closed	Staff do not have one reliable way to save a problem, see who owns it, and learn what happened next.	Treat as a wider program issue: an app failure and the same connectivity problem raised three times without action show both a tool problem and a follow-up problem.

6.2 Program Learning Note

Question 2: What short learning note should be shared, and what one process change would help capture feedback better?

Recent field feedback shows that several small design and support gaps are reducing session quality. Materials do not always match the ward setting, and some content repeats information families have already seen. Attendance changes with ward routines and Ramadan, while current participation rules may leave out interested family members. Nurse transfers also leave new staff without enough support. At the same time, staff are losing data in the app and do not know whether problems raised on calls are being acted on. I recommend one shared offline feedback and action log linked to the monthly monitoring form. Each issue should record the site, theme, urgency, owner, due date, and closure note. The Monitoring and Implementation teams should review the log together each month and send the final action back to the person who raised it.

Word count: 135.

7 Exported data workbook

The cleaned source data, comparison tables, results, and data checks are saved in `Updated_Data.xlsx`.

8 What the data cannot tell us

- The files do not say which quarter was reported or the exact period behind the older averages. The quarter estimates are only a rough comparison, not a prediction.
- Most district facility counts differ between the two files. I could not settle these differences because the files do not share a fixed facility ID or dated facility list.
- City rows were grouped with their parent districts only when comparing the two files. The original monitoring locations were kept.

- I kept the rounded averages from the source files. Session and participant totals were added again from the detailed columns so the maths could be checked.
- Every cleaning step and calculation is in this R Markdown file. No cleaned value was changed by hand outside the code.

9 References used for the practical recommendations

- World Health Organization. Toolkit for Routine Health Information Systems data.
- SurveyCTO. Collecting data online and offline.